

Name: Daniel S May | DOB: 10/11/1989 | MRN: 10105654130 | PCP: Joshua W Clark, NP | Legal Name:
Daniel S May

Office Visit - Oct 03, 2025

with Gregory Krause, MD at Mass Eye and Ear Otolaryngology Clinic



Notes from Care Team



Progress Notes by Gregory Krause, MD at 10/3/2025 1:30 PM



Mass General Brigham
Mass Eye and Ear



HARVARD
MEDICAL SCHOOL

DEPARTMENT--OTOLARYNGOLOGY MASSACHUSETTS EYE AND EAR INFIRMARY

Date: 10/3/2025

Patient: Daniel S May DOB: 10/11/1989 MEEI MRN: 8026121

PCP: Pcp, Not Required

History of Present Illness

Daniel S May is a 35 year old male with chronic sinus issues who presents with bilateral ear fullness and eustachian tube dysfunction. He was referred by Dr. Gray for treatment of otologic symptoms.

He has a history of chronic sinus issues and has undergone two septoplasty procedures over the past ten years. He experiences chronic ear infections approximately every month or two, which have worsened over the past year. His current symptoms include ear fullness, popping, and crackling, particularly in the left ear. He describes a sensation of a sticky, sappy substance that he feels may be present in his eustachian tubes.

He has been using budesonide nasal rinses, which have cleared his sinuses but not resolved the ear symptoms. He has been on oral

prednisone in the past, which also did not alleviate his ear symptoms.

He experiences occasional migraines, with two severe episodes in the past few months, but denies frequent ear pain. He has not flown recently but recalls experiencing vertigo after a flight ten years ago. No autophony.

9/17/25 Rhinology visit-

Impression:

1. Rhinitis medicamentosa - improved.
2. Chronic rhinosinusitis.
3. Inferior turbinate hypertrophy.

Plan:

Mr. May and I reviewed his history and symptoms and the findings on nasal endoscopy. Overall his nasal symptoms have improved since stopping the hydrogen peroxide in his rinses and minimizing the Afrin and starting the Budesonide irrigations. I stressed the importance of complete cessation of all topical nasal decongestant sprays. He can continue the Budesonide irrigations and can try to decrease to daily. I do think that allergy evaluation will be useful for him and we will help arrange this. Most of his symptoms are currently ear related and we will try to help expedite his Comprehensive ORL evaluation for this.

8/11/25 MEE ED visit-

Daniel S May (MRN 8026121) is a 35 y.o. male with PMH including CRS s/p sinus surgery x2 >10 years ago. Was seen in MEE ED 07/16/25 with months of sinus congestion and nasal obstruction. Started on inhaled steroid nasal sprays and referred to Rhinology.

Had scheduling errors and returns to the ED for follow up.

Symptoms include thick nasal discharge, facial pressure,

Has had recurrent sinus infections in the past.

Has used saline irrigations, flonase without improvement.

ASSESSMENT

CRS w/o NP- CT obtained today showing bilateral sinus disease.

Recommended starting nasal saline irrigations and flonase BID. Referral to Rhinology

Left ear clicking- differential includes ETD vs middle ear myoclonus. Obtain audiogram, follow up with neurotology

PAST MEDICAL HISTORY

Past Medical History:

Diagnosis

Date

- Trauma to eye

MEDICATIONS/ALLERGIES

Current Outpatient Medications:

- budesonide (PULMICORT) 0.5 mg/2 mL nebulizer solution, Add one vial to 240ML neilmed sinus rinse and rinse nasally daily, Disp: 60 mL, Rfl: 11, Last Dispense: Unknown (outside pharmacy)
- clonazepam (KLONOPIN) 1 MG tablet, Take 1 mg by mouth 2 (two) times a day as needed for anxiety., Disp: , Rfl: , Last Dispense: Unknown (patient-reported)
- fluticasone propionate (FLONASE) 50 mcg/actuation nasal spray, 1 spray by Nasal route daily., Disp: 30 mL, Rfl: 0, Last Dispense: Unknown (outside pharmacy)
- naproxen (NAPROSYN) 500 MG tablet, Take 1 tablet (500 mg total) by mouth 2 (two) times a day with meals for 15 days., Disp: 30 tablet, Rfl: 0, Last Dispense: Unknown (outside pharmacy)
- oxycodone 5 MG immediate release tablet, Take 1 tablet (5 mg total) by mouth every 6 (six) hours as needed. (Patient not taking: Reported on 7/16/2025), Disp: 15 tablet, Rfl: 0, Last Dispense: Unknown (outside pharmacy)
- prednisone (DELTASONE) 10 MG tablet, 50 mg x 3 days 40MG x3days 30MGx3 days 20MG x3days 10MG x 3days, Disp: 45 tablet, Rfl: 0, Last Dispense: Unknown (outside pharmacy)

he has no known allergies.

SOCIAL HISTORY

he reports that he has never smoked. He has never used smokeless tobacco. He reports that he does not currently use alcohol. He reports that he does not currently use drugs.

Family History

Problem

Relation

Age of Onset

- | | | |
|------------------------|--------|--|
| • Diabetes | Neg Hx | |
| • Glaucoma | Neg Hx | |
| • Macular degeneration | Neg Hx | |

EXAMINATION

There were no vitals taken for this visit.

GENERAL APPEARANCE: well developed, well nourished patient in no acute distress.

VOICE: normal

BREATHING: normal, not labored, no stridor

FACE: no lesions

FACIAL STRENGTH: normal and symmetric

AD: pinna normal, no lesions or erythema. Canal clear. TM clear

AS: pinna normal, no lesions or erythema. Canal clear. TM clear

NOSE: Mucous membranes moist, no lesions, polyps, purulent discharge or bleeding

Septum: midline

Inferior Turbinates: normal size

ORAL CAVITY: buccal mucosa moist, no lesions.

Tongue no lesions

Hard palate normal, no masses.

OROPHARYNX: Soft palate normal, no lesions, normal elevation

Tonsils: 0

Posterior pharyngeal wall: No cobblestoning

NECK: Trachea midline. No palpable lymphadenopathy.

No masses.

SALIVARY: Parotid glands normal, no tenderness, swelling or masses.

Submandibular glands normal size and shape, no tenderness.

.Procedure: Examination of the Ear Under the Microscope

In order to further assess findings identified on otoscopy, binocular microscopy was performed.

Procedure discussed with patient and verbal consent obtained.

Patient placed under the microscope.

AD: Canal clear. TM clear.

Retracted TM: No

Movement of TM with ipsilateral nasal breathing: No
TM movement with modified Valsalva

AS: Canal clear. TM Clear

Retracted TM: No

Movement of TM with ipsilateral nasal breathing: No
TM movement with modified Valsalva

Well tolerated, no bleeding or pain.

ASSESSMENT/PLAN

Bilateral aural fullness

Symptoms greater in left ear. Most likely secondary to eustachian tube dysfunction given concurrent sinus issues and lack of autophony, TMJ, Migraine, or vertiginous symptoms. Will obtain audiogram/tympanogram. Discussed with Mr. May that if negative pressure noted on tympanogram would recommend myringotomy tube insertion in left ear. If no negative pressure, would recommend a diagnostic tympanostomy and if aural fullness symptoms resolved than perform a subsequent PE tube insertion.

Gregory E. Krause, MD
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